GPSC CBRT · HOMOEOPATHIC PHYSICIAN · CORE

Topic 4: Repertory & Repertorisation

Complete notes with pictures, memory hooks, rapid revision and practice MCQs.

Made with GPSCavy · raghav511.github.io/gpsc-avy · 23 October 2026

Topic 4: Repertory & Repertorisation

How to use this guide

Repertory is one of the ten Core topics (the Core section carries 25 of the 60 CBRT marks), so expect about 2 to 3 questions. The GPSC syllabus splits it into four headings, and this guide follows them in order:

1. **4.1 History and structure:** from Boenninghausen to Kent, then synthetic and computer repertories (RADAR, Synthesis).
2. **4.2 Types:** General, Purposive/Particular, Clinical and Card.
3. **4.3 Methods:** Kentian, Boenninghausen totality and concordance, elimination, rubric conversion.
4. **4.4 Symptom evaluation:** the hierarchy and rubric differentiation.

- Each section ends with a coloured **memory hook**: say it aloud once, and it will carry the facts.
- The pictures are drawn to be recalled as pictures.
- The one-glance recall card near the end is for the last ten minutes before the exam.
- Standard books: S. K. Tiwari, *Essentials of Repertorisation*; Kent's *Repertory* (6th American edition); Boenninghausen's *Therapeutic Pocket Book* (T. F. Allen edition); Boger's *BBCR*.
- The fuller rapid-revision doc you already have, *Homoeopathic Repertory – Rapid Revision*, stays unchanged and goes deeper on rubric locations.

4.1 History and structure of repertories

- A **repertory** (Latin *repertorium*, a storehouse where things can be found) is an index of the symptoms of the Materia Medica.
- It is a means to an end: the final choice is always confirmed in the Materia Medica.
- A **rubric** (Latin *rubrica*, red) is a heading; old books printed headings in red ink.

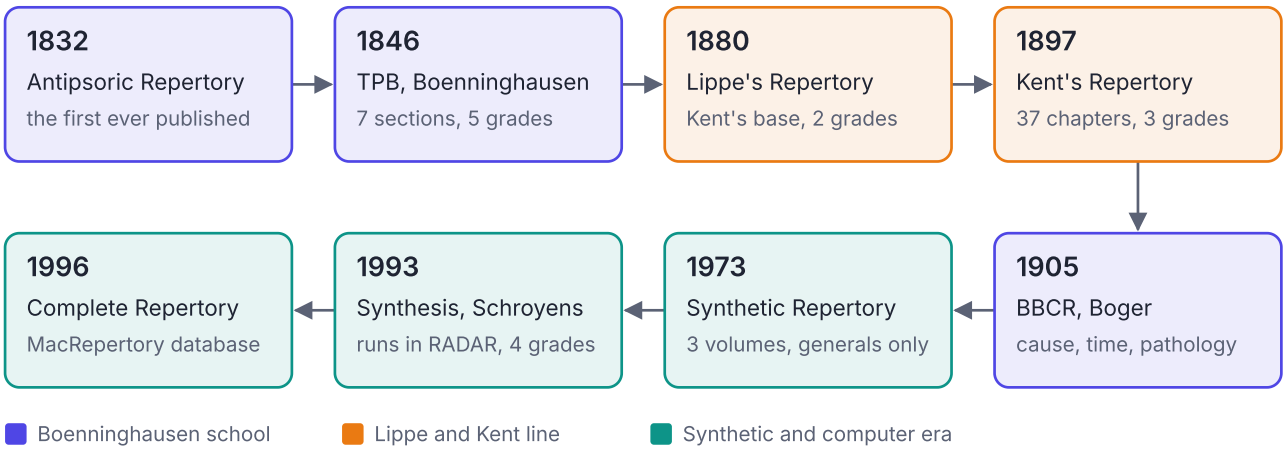
The timeline

Year	Work	Author	Exam hook
1805	*Fragmenta de Viribus*, Part 2 (index)	Hahnemann	First repertory-like index
1832	Repertory of the Antipsoric Medicines	Boenninghausen	First published repertory
1835	Repertory of the Non-Antipsoric Medicines	Boenninghausen	Companion volume

Year	Work	Author	Exam hook
1846	Therapeutic Pocket Book (TPB)	Boenninghausen	7 sections, 5 grades
1880	Repertory to the More Characteristic Symptoms	C. Lippe	Base for Kent; 2 grades
1890–92	Concordance Repertory (6 vols)	W. D. Gentry	Puritan, key-word index
1891	TPB English edition	T. F. Allen	342 remedies
1896–97	Repertory of Hering's Guiding Symptoms	C. B. Knerr	48 chapters, 4 grades
1897–99	Repertory of the Homoeopathic MM	J. T. Kent	37 chapters, 3 grades
1904	Clinical Repertory	J. H. Clarke	Clinical type
1905	BBCR	C. M. Boger	Causation, time, pathological generals
1915	Synoptic Key	C. M. Boger	Basis for Phatak
1973–74	Synthetic Repertory (3 vols)	Barthel and Klunker	Mind; Generals; Sleep, Dreams, Sex
1993	Synthesis	F. Schroyens	RADAR; 4 grades
1996	Complete Repertory	R. van Zandvoort	MacRepertory

Repertories grew from Boenninghausen (1832) to computer databases (1996)

Read the top row left to right, then follow the arrow down and back



repertory timeline · 8 milestones, 3 schools

Recall the picture as a snake: two Boenninghausen boxes, two Kent-line boxes, Boger turning the corner, then the three modern books.

The big three

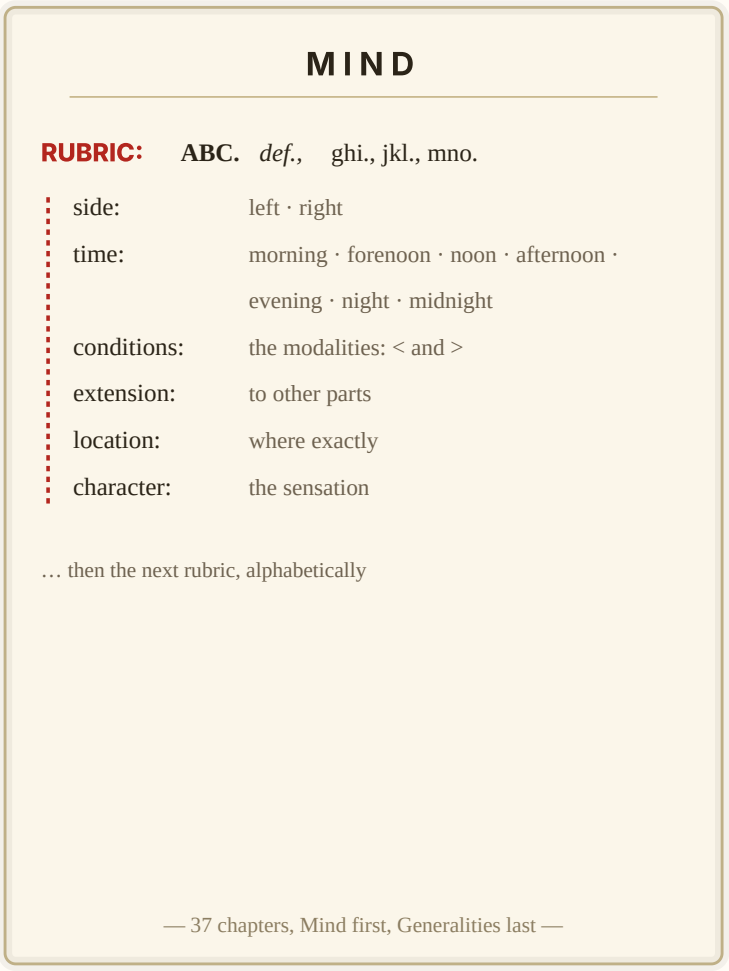
- **Boenninghausen's TPB (1846).** It has 7 sections: Mind and intellect; Parts of the body; Sensations and complaints; Sleep and dreams; Fever; Aggravations and ameliorations (modalities); Relationship of remedies (concordance).
- It has 5 grades and about 125 remedies in the original (342 in Allen's edition).
- Its ideas are the **complete symptom** (Location + Sensation + Modality + Concomitant), **grand generalisation** (a modality found in one part belongs to the whole patient) and the **doctrine of analogy** (fill a gap from similar symptoms elsewhere).

BBCR (C. M. Boger, 1905). *Boger Boenninghausen's Characteristics and Repertory* is a Materia Medica plus a repertory on Boenninghausen's plan, with more particular rubrics. Boger added **causation, time and pathological generals** (for example a tendency to suppuration or haemorrhage).

- **Kent's Repertory (1897–99).** Built on Lippe's repertory, Kent's has 37 chapters from **Mind** to **Generalities** in Hahnemann's head-to-foot order.
- It has 3 grades: **bold = 3**, *italics = 2*, roman = 1.
- The plan runs from general to particular (Swedenborg's influence: the inner is above the outer).
- Within a rubric, sub-rubrics run **Side → Time → Conditions (modalities) → Extension → Location → Character (sensation)**.
- Times run morning, forenoon, noon, afternoon, evening, night, midnight.
- Desires, aversions and thirst sit in **Stomach**; food aggravations sit in **Generalities**.
- Editions: 1st 1897; 2nd 1908; 3rd 1924, after Kent's death in 1916; 6th American 1957.

Kent's Repertory: how a page works

Grades by typeface; sub-rubrics in a fixed order



Reading a page of Kent

GRADES

ABC.	bold = 3 marks
def.	italic = 2 marks
ghi.	roman = 1 mark

SUB-RUBRIC ORDER

1. Side
2. Time
3. Conditions
4. Extension
5. Location
6. Character

Plan: general to particular,
head to foot (Hahnemann).
Desires, aversions, thirst: Stomach.
Food aggravations: Generalities.

Remedy names on the page are placeholders.

Kent's Repertory · grades by typeface, sub-rubrics in a fixed order

Modern and computer repertories

- **Synthetic Repertory** (Barthel and Klunker) covers only general symptoms, compiled from many sources: Vol I Mind, Vol II Generals, Vol III Sleep, Dreams and Sexuality.
- **Synthesis** (Schroyens) is Kent's 6th edition corrected and enlarged, with author references and 4 grades. It runs in **RADAR** (Rapid Aid to Drug Aimed Research; Archibel, Belgium).
- **Complete Repertory** (van Zandvoort) is the database in **MacRepertory** (Kent Homeopathic Associates, USA).
- **Hompath** was developed in India by Dr Jawahar Shah. **ISIS** is by David Witko.
- Computers give speed, many repertories at once and many analysis strategies. But they are only as good as the rubrics chosen: garbage in, garbage out.

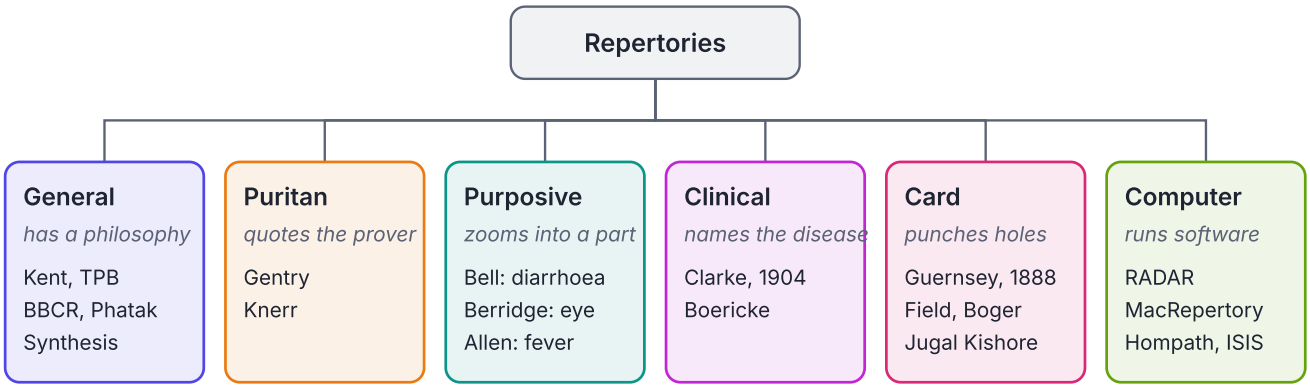
Memory hook: *"Boenninghausen built the pocket, Boger filled it with time, Kent climbed from Mind to Generalities."* Then the numbers in a chant: **TPB 7 and 5, Kent 37 and 3, Knerr 48 and 4, Gentry 6 volumes.**

4.2 Types of repertories

The syllabus groups repertories as **General**, **Purposive/Particular**, **Clinical** and **Card**. MCQs usually name a book and ask its type, or name a type and ask for the odd one out.

Type	What it is	Examples to remember
General (logical / philosophical)	Covers the whole Materia Medica and all body parts, built on a philosophy	TPB, BBCR, Kent, Boger's Synoptic Key, Phatak, Synthesis
Puritan	Keeps the provers' exact words, with no generalising	Gentry's Concordance, Knerr
Purposive / Particular / Regional	One region, one disease or one symptom group	Bell (Diarrhoea); H. C. Allen (Intermittent fever); Berridge (Eye); Minton (Uterine therapeutics); Douglass (Skin)
Clinical	Rubrics are disease names	Clarke's *Clinical Repertory*; Boericke's repertory (with the *Pocket Manual*)
Card (mechanical)	Each card is one rubric, punched at fixed remedy positions	Guernsey (first, 1888); Field; Boger; Jugal Kishore (India)
Computer	Software that runs repertory databases	RADAR, MacRepertory, Hompath, ISIS

Six types of repertory: one verb tells each apart



types of repertory · 6 branches with examples

Read each branch as a verb: if an MCQ names a book, ask what it does (plan, quote, zoom, name, punch or compute) and the type follows.

By arrangement

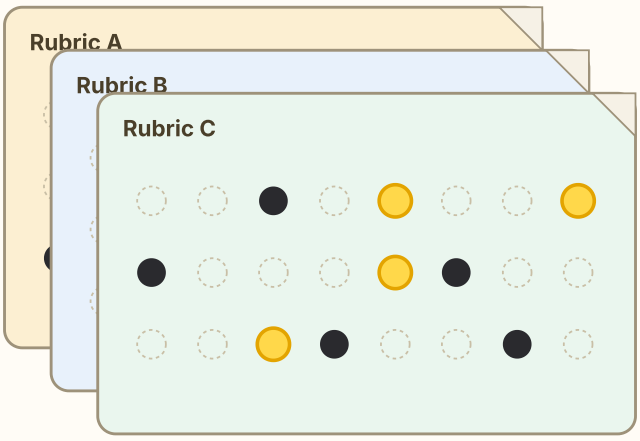
- **Schematic or anatomical** (Hahnemann's head-to-foot order): Kent, Knerr, BBCR.
- **Alphabetical**: Gentry (by key word), Phatak (by rubric), Murphy (by chapter), Clarke.
- The classic two-way split is the **logico-utilitarian** group (symptoms reshaped to fit the book's plan: Kent, TPB, BBCR) against the **Puritan** group (Gentry, Knerr).

How a card repertory works

- Each card carries one rubric, and each remedy has a fixed numbered position on every card.
- A hole is punched where the remedy is present.
- Stack the cards for the chosen rubrics and hold them to the light: holes that pass through every card are the remedies common to all rubrics.
- It is quick and needs no counting.
- Its limits: it ignores grades, and one small rubric can wrongly drop the true remedy.
- **Marcos Jimenez** first brought grading into cards; **Field's** cards are based on Kent.

Card repertory: let the light decide

Punched cards, stacked: the light passes only where every rubric agrees



Hold the stack to the light: it shines only through holes punched in every card.

How a card repertory works

One card = one rubric.
Every remedy has a fixed, numbered position on every card; a hole is punched where the remedy is in the rubric.

Light through every card = remedy common to all rubrics

LIMITS

Ignores grades; one small rubric can wrongly drop the true remedy.

NAMES

Guernsey (first, 1888); Field (based on Kent); Boger; Jugal Kishore (India). Marcos Jimenez first graded cards.

Card repertory · holes that line up through every card mark the common remedies

Clinical and regional repertories, in one line each

- **Clinical:** quick in acute and one-sided cases, but prescribing on the disease name ignores individualisation, so it should never be the only basis. G. H. G. **Jahr** is called the father of the clinical repertory.
- **Regional:** useful when one region dominates and generals are scanty; its remedy lists for that field are richer than a general repertory's.

Memory hook: *"Generals Plan, Puritans Quote, Purposives Zoom, Clinics Name, Cards Punch."*
One verb per type: the general repertory **plans** (a philosophy), the Puritan **quotes** the prover, the purposive **zooms** into one part, the clinical **names** the disease, and the card **punches** holes.

4.3 Methods of repertorisation and rubric conversion

Repertorisation means finding the remedy, or the small group of remedies, that covers the totality of the case with the help of a repertory. The final choice is made by studying the shortlist in the Materia Medica.

The steps, in order

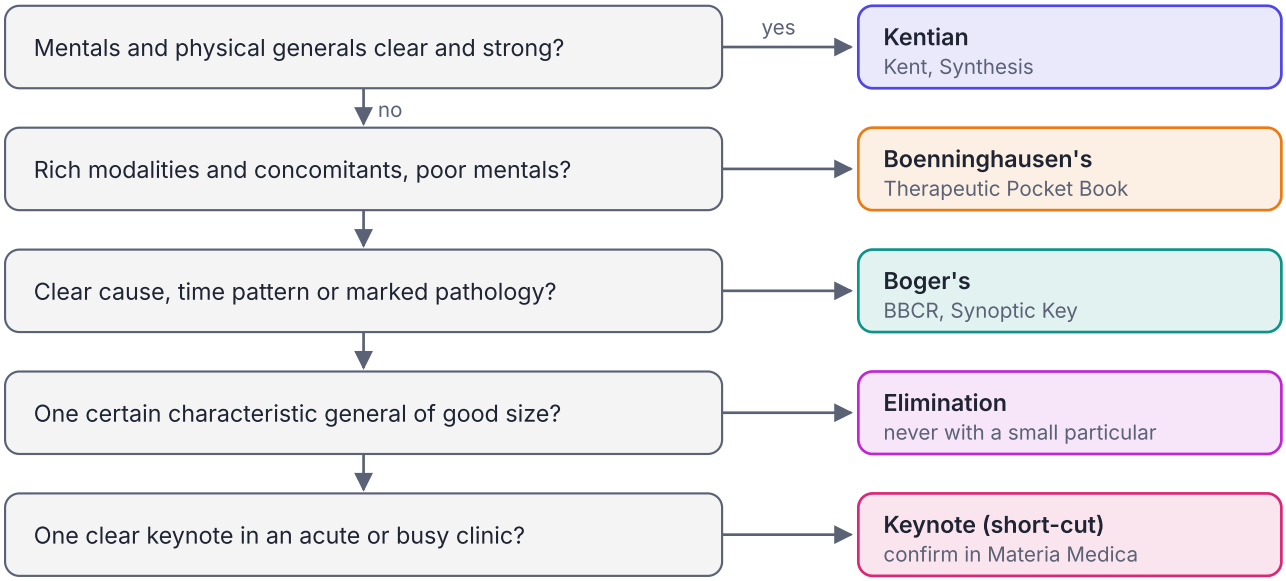
1. Case taking (recording)
2. Analysis of the case (classify the symptoms)
3. Evaluation of symptoms (grade their importance)
4. Erecting the repertorial totality
5. Choosing the repertory and method that suit the case
6. Rubric conversion (patient's words into repertory language)
7. Repertorisation (working out the rubrics)
8. Repertorial result, then differentiation in the Materia Medica, then the prescription

Which method for which case

Method	Core idea	Best when	Repertory
Kentian (totality)	Mental generals, then physical generals, then particulars	Chronic cases with clear mentals and generals	Kent, Synthesis
Boenninghausen's	Complete symptom (L-S-M-C), generalised modalities, concomitants	Rich modalities and concomitants, poor mentals	TPB
Boger's	Causation and time first, pathological generals, then modalities and mentals	Clear cause and time pattern; marked pathology	BBCR, Synoptic Key
Elimination	One strong rubric drops every remedy not in it	One or two very reliable characteristic generals	Any
Keynote (short-cut)	One or two keynotes, confirmed in MM	Clear keynotes; acute or busy practice	Any; clinical
Mechanical (card)	Superimpose punched cards	Quick screening	Card repertories
Computer	Software combines rubrics by chosen strategies	Large cases	RADAR, MacRepertory, Hompath

Pick the method from what the case is rich in

Ask top to bottom; the first yes gives the method. Every result is confirmed in the Materia Medica.



method chooser · 5 questions, 5 methods

In a case-based MCQ, find what the case is rich in, walk down the ladder and stop at the first yes.

Kent's working rules

- Take the strongest mental and physical generals first, then the particulars that are peculiar.
- If no exact rubric exists, go to the larger general rubric: Kent's book is built to be used general first.
- There are two ways to count: **numerical** (how many rubrics a remedy covers) and **qualitative** (the sum of grades 3, 2 and 1). The totals guide the choice; they do not dictate it.

Elimination: the trap MCQs test

- The eliminating rubric should be a **characteristic general** that is certain and of reasonable size.
- Never eliminate with a small particular rubric or a doubtful symptom: the true remedy may be missing only because the rubric is incomplete.
- **Cascading elimination** means each rubric eliminates for the next, all through the exercise.

Rubric conversion

Rubric conversion turns the patient's language into the repertory's language without changing the meaning.

- "I feel worse when people try to comfort me" becomes *Mind – Consolation – aggravates*.
- "I am scared to be alone at home" becomes *Mind – Fear – alone, of being*.
- "My headache is better when I press hard" becomes *Head – Pain – pressure – amel*.

Use synonyms and cross-references, choose the rubric that matches the exact meaning, and when unsure take the broader rubric. Know where things sit: in Kent, thirst and food desires are in Stomach.

Memory hook: *"Kent climbs down, Boenninghausen spreads out, Boger asks why and when."* Kent goes from the top (mind) down to the parts; Boenninghausen spreads one modality over the whole patient; Boger starts with the **cause** and the **time**.

4.4 Symptom evaluation and rubric differentiation

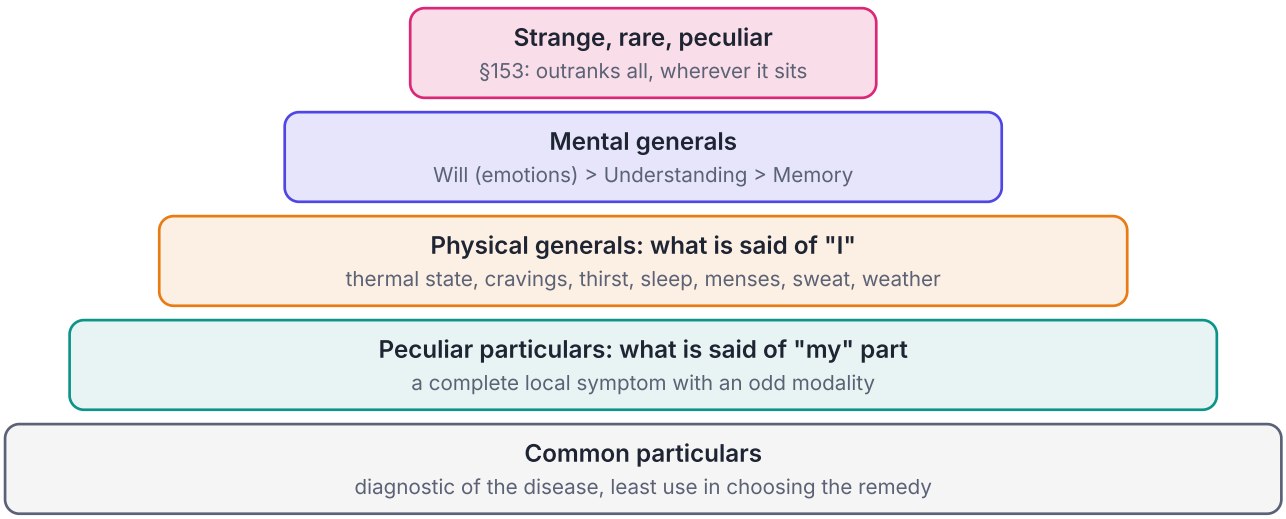
Evaluation decides which symptoms lead the repertorisation. The most-tested idea: characteristic (strange, rare, peculiar) symptoms rank highest and common diagnostic symptoms rank lowest (Organon §153).

Kent's hierarchy, high to low

1. **Mental generals**, in their own order: **Will** (emotions: loves, hates, fears, desires) above **Understanding** (intellect: delusions, confusion) above **Memory**.
2. **Physical generals**, the things the patient says of "I": thermal state, appetite, cravings and aversions, thirst, sleep and dreams, sexual sphere and menses, perspiration, time and weather modalities.
3. **Particulars**, the things said of a part: peculiar particulars count; common particulars count least.

The higher a symptom sits, the more it decides the remedy

Kent's order of evaluation, with the strange, rare and peculiar symptom on top



symptom hierarchy · 5 levels, high to low

Picture a pyramid: the narrow top decides the remedy, the wide base only names the disease.

A strange, rare and peculiar symptom outranks this order wherever it sits.

Symptom classes in one line

Term	Meaning
General	Refers to the whole person ("I am chilly")
Particular	Refers to one part ("my knee is worse in cold")
Common	Found in most patients with the disease; diagnostic, low value
Characteristic	Individualising: striking, singular, peculiar (§153)
Basic and determinative	Garth Boericke's terms: disease-defining versus remedy-deciding
Concomitant	Accompanies the chief complaint with no pathological link (Boenninghausen)
Complete symptom	Location + Sensation + Modality (+ Concomitant)
Keynote	A highly characteristic symptom pointing strongly to one remedy
Eliminating symptom	A strong general used to narrow the remedy list
Pathological general	Pathology affecting the whole system, such as a tendency to suppuration (Boger)
Negative general	An expected general that is absent, such as thirstlessness in fever

Who ranks what first

- **Boenninghausen:** the complete symptom; concomitants are most valuable; modalities can be generalised.
- **Kent:** mentals first, then physical generals, then particulars.
- **Boger:** causation and time first, then pathological generals, then modalities and mentals.

Rubric differentiation (Kent)

- **Consolation aggravates** (Nat-m, Sep, Ign, Sil) against **Consolation ameliorates** (Puls, Phos).
- **Anxiety** is vague uneasiness, **Fear** is of a definite thing, **Anguish** is extreme anxiety.
- **Irritability** (easily annoyed), **Anger** (an outburst) and **Violent** are three different rubrics.
- **Forsaken feeling** is a Mind rubric; *Delusion*, *deserted* is a false belief.
- A **delusion** is a fixed false idea; **fancies** are imaginations the patient knows are not real.
- *Right side* in a particular chapter refers to that part; *Sides*, *right* in Generalities refers to the whole body.

Memory hook: *"Will before Wit before Memory; I before My."* Emotions outrank intellect, which outranks memory; what the patient says of **I** (a general) outranks what is said of **my** knee (a particular). And **SRP beats them all**.

Rapid revision

Cue	Fact
First published repertory	Boenninghausen, *Repertory of the Antipsoric Medicines*, 1832
TPB	1846; 7 sections; 5 grades; about 125 remedies (342 in Allen's 1891 edition)
Complete symptom	Boenninghausen: Location, Sensation, Modality, Concomitant
Grand generalisation	TPB: a modality of one part is taken as true of the whole patient
BBCR	C. M. Boger, 1905; causation, time, pathological generals
Kent's Repertory	1897–99; 37 chapters; Mind first, Generalities last; 3 grades
Kent's base	Lippe's repertory (2 grades)
Chapter after Mind	Vertigo
Food desires and thirst (Kent)	Stomach chapter
Knerr	Index to Hering's *Guiding Symptoms*; 48 chapters; 4 grades
Gentry	*Concordance Repertory*, 6 volumes, Puritan, alphabetical key words
Clarke	*Clinical Repertory*, 1904
First card repertory	W. J. Guernsey, 1888
Synthetic Repertory	Barthel and Klunker; 3 volumes (Mind; Generals; Sleep, Dreams, Sex)
Synthesis	Schroyens, 1993; RADAR; 4 grades
Complete Repertory	van Zandvoort; MacRepertory
Best eliminating rubric	A characteristic general of good size, never a small particular
Kent's top mental	Will (emotions), then Understanding, then Memory
Least valuable symptom	Common particular

Cue	Fact
Basic and determinative	Garth Boericke

One-glance recall card

Topic 4 recall card: the 15 facts asked most

Cover the small lines, say the fact from the big cue, then check

1832
first published repertory:
Boenninghausen, Antipsoric

TPB 7 · 5
1846: 7 sections, 5 grades;
~125 remedies, 342 in Allen

L + S + M + C
complete symptom:
Boenninghausen's idea

Grand generalisation
a modality of one part
belongs to the whole patient

BBCR 1905
Boger: causation, time,
pathological generals

Kent 37 · 3
Mind first, Generalities last;
bold 3, italic 2, roman 1

Lippe
Kent's base repertory,
2 grades (1880)

Vertigo
the chapter right after
Mind in Kent

Stomach
Kent: desires, aversions
and thirst sit here

Knerr 48 · 4
index to Hering's
Guiding Symptoms

Gentry 6 vols
Concordance, Puritan,
alphabetical key words

Guernsey 1888
first card repertory;
Jugal Kishore in India

Synthesis
Schroyens 1993; RADAR;
Complete Rep. = MacRepertory

Will > Wit > Memory
Kent's order inside
the mental generals

Eliminate wisely
use a big characteristic
general, never a small particular

Topic 4 recall card · 15 tiles

For the last ten minutes before the exam: cover the small lines of each tile and say the fact from the big cue.

Practice MCQs

1. The first published repertory in homoeopathy was: (a) Kent's Repertory (b) the Therapeutic Pocket Book (c) the Repertory of the Antipsoric Medicines (d) Knerr's Repertory
2. The number of sections in Boenninghausen's Therapeutic Pocket Book is: (a) 5 (b) 7 (c) 9 (d) 37
3. Kent's Repertory grades remedies in: (a) 2 grades (b) 3 grades (c) 4 grades (d) 5 grades
4. The last chapter of Kent's Repertory is: (a) Skin (b) Sleep (c) Generalities (d) Perspiration
5. Kent's Repertory was developed from the repertory of: (a) Jahr (b) Lippe (c) Gentry (d) Knerr

6. Knerr's Repertory is an index to: (a) Allen's Encyclopedia (b) Hering's Guiding Symptoms (c) Hahnemann's Chronic Diseases (d) Clarke's Dictionary
7. A Puritan repertory is: (a) Kent (b) BBCR (c) Gentry's Concordance (d) Phatak
8. The concept of the complete symptom was introduced by: (a) Kent (b) Boger (c) Boenninghausen (d) Hering
9. Grand generalisation is a feature of: (a) Kent (b) the TPB (c) Knerr (d) Clarke
10. BBCR gives special place to: (a) causation, time and pathological generals (b) only mentals (c) only keynotes (d) disease names
11. In Kent's Repertory, desire for sweets is found in the chapter: (a) Generalities (b) Mind (c) Stomach (d) Mouth
12. The first card repertory was prepared by: (a) Jugal Kishore (b) Boger (c) W. J. Guernsey (d) Field
13. Bell's repertory deals with: (a) intermittent fever (b) diarrhoea (c) the eye (d) the uterus
14. Clarke's repertory is classified as: (a) Puritan (b) Clinical (c) Card (d) General logical
15. Synthesis repertory is linked with the software: (a) Hompath (b) RADAR (c) MacRepertory (d) ISIS
16. The highest rank among mental generals in Kent's evaluation goes to: (a) memory (b) intellect (c) will and emotions (d) delusions
17. The least valuable symptoms in repertorisation are: (a) mental generals (b) common particulars (c) physical generals (d) peculiar symptoms
18. The best eliminating symptom is: (a) a small particular rubric (b) a common symptom (c) a characteristic general of good size (d) a doubtful symptom
19. Converting the patient's language into repertory language is called: (a) repertorisation (b) rubric conversion (c) case analysis (d) elimination
20. The method best for a case rich in modalities and concomitants but poor in mentals is: (a) Kentian (b) Boenninghausen's (c) keynote (d) card

Answer key

Q	Answer	Why
1	c	Boenninghausen, 1832; the TPB came in 1846
2	b	7 sections, from Mind to Relationship of remedies
3	b	Bold 3, italics 2, roman 1
4	c	Mind first, Generalities last
5	b	Lippe's repertory of 1880, with 2 grades

Q	Answer	Why
6	b	48 chapters, 4 grades
7	c	Keeps the provers' exact words
8	c	Location, Sensation, Modality, Concomitant
9	b	A local modality is generalised to the whole patient
10	a	Boger's additions to Boenninghausen's plan
11	c	Kent places desires, aversions and thirst in Stomach
12	c	Guernsey, 1888, "Boenninghausen's Slips"
13	b	A purposive repertory on diarrhoea
14	b	Rubrics are disease names
15	b	Schroyens' Synthesis runs in RADAR
16	c	Will, then Understanding, then Memory
17	b	Common to the disease, so they do not individualise
18	c	A small rubric may wrongly drop the true remedy
19	b	Same meaning, repertory wording
20	b	The TPB generalises modalities and values concomitants